

RIVERGATE REGIONAL TRAUMA CENTER

Level I Trauma Center

2100 Medical Center Drive, Rivergate, CA 92550

Reyes, Yolanda

MRN: RRTC-0074318 DOB: 06/22/1991 (34 yrs) Sex: F

Admission Date: 10/18/2025 C: (inpatient) Coverage: Medi-Cal (BIC on file) / BlueShield of California — HMO

TRAUMA ADMISSION — HISTORY & PHYSICAL**Date/Time of Admission:** 10/18/2025 02:45**Admitting Service:** Trauma Surgery / Surgical ICU**Attending:** Priya Ramaswamy, MD — Trauma Surgery**Mode of Arrival:** HEMS (rotor-wing) airlift from scene**Scene:** Intersection of South Main Street and Third Avenue, Rivergate**Mechanism:** Pedestrian struck by transit bus**Emergency Contact:** Marisela Reyes (sister) — relationship on file; on route to hospital**CHIEF COMPLAINT**

Unresponsive pedestrian struck by transit bus.

HISTORY OF PRESENT ILLNESS

Ms. Yolanda Reyes is a 34-year-old female (DOB 06/22/1991) pedestrian struck by an Alder Creek Transit District bus at approximately 02:05 on 10/18/2025 while standing at a bus stop on South Main Street just north of Third Avenue. Per HEMS and ground-EMS report, the transit bus left its lane at an estimated speed of approximately 30 mph, first striking a City street-light standard and then striking the patient at the curbside. Bystanders and first responders describe the patient as having been thrown approximately fifteen feet from the point of impact and coming to rest on the roadway. She was found prone and unresponsive at the scene.

Initial ground-EMS assessment documented GCS 5 (E1 V1 M3), agonal respirations with obvious airway compromise, and multiple gross deformities of the pelvis and both lower extremities. Rapid-sequence intubation was performed in the field with a 7.0 endotracheal tube secured at 22 cm at the lip; end-tidal CO₂ confirmed placement. Bilateral 16-gauge IV access established; 1L crystalloid infused en route. C-collar, long-board, and pelvic binder applied. HEMS launched from the scene at approximately 02:25 with airlift to this trauma center; arrival to the trauma bay at 02:45.

No prior history available directly from the patient given her neurologic status. Emergency contact (sister Marisela Reyes) reports the patient is otherwise generally healthy, works as a retail sales associate in Rivergate, takes no chronic medications known to family, no known drug allergies, and has no prior surgeries. Family denies known bleeding disorders. Immunization and prior medical history to be obtained upon family arrival and record review.

PAST MEDICAL / SURGICAL / SOCIAL HISTORY

- **PMH:** None known per family
- **PSH:** None known per family
- **Medications:** None reported
- **Allergies:** NKDA per family
- **Social:** Lives alone; sister nearby. Non-smoker per family. Alcohol / substance use per family: occasional social use only.
- **Family Hx:** Non-contributory per family at time of admission.

VITAL SIGNS ON ARRIVAL (Trauma Bay, 02:45)

Parameter	Value
Temperature	35.4 °C (rectal)
Heart Rate	128 bpm, sinus tachycardia
Blood Pressure	88/54 mmHg
Respirations	Mechanically ventilated, AC 16, TV 450, FiO ₂ 1.0
SpO ₂	94% on 100% FiO ₂
GCS	5T (E1 V1T M3), sedated post-intubation
Pupils	4 mm R / 3 mm L, sluggish bilaterally

PRIMARY SURVEY

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- **A (Airway):** Intubated in field; ETT confirmed by end-tidal CO₂, bilateral chest rise, and post-arrival CXR. C-collar in place.
- **B (Breathing):** Diminished breath sounds on the right with palpable subcutaneous emphysema over the right chest wall; crepitus over right lateral ribs. Left side clear. Concern for right hemopneumothorax; right-sided 32 Fr chest tube placed in trauma bay with return of 350 mL blood and immediate air leak — lung re-expanded on repeat CXR.
- **C (Circulation):** Hypotensive and tachycardic. Two large-bore peripheral IVs supplemented by right femoral cordis. Massive transfusion protocol activated at 02:52. FAST examination positive in the left upper quadrant (perisplenic free fluid); pericardial and pelvic views negative. Pelvic binder in place from the field.
- **D (Disability):** GCS 5T; pupils as above. No spontaneous movement of extremities.
- **E (Exposure):** Fully exposed and warmed with forced-air blanket. Obvious open wounds noted over the pelvis and both lower legs.

SECONDARY SURVEY

- **HEENT:** Boggy left parietal scalp hematoma with overlying laceration approximately 6 cm, oozing. No obvious skull step-off. Hemotympanum absent. Facial bones stable to palpation. Pupils as documented. No CSF rhinorrhea or otorrhea.
- **Neck:** In collar; no gross deformity; no tracheal deviation; no obvious cervical bruit.
- **Chest:** Right-sided chest wall crepitus and ecchymosis spanning the right 4th–9th ribs anteriorly and laterally; palpable flail segment right lateral. Left chest wall intact. Sternum stable.
- **Abdomen:** Distended, tense; diffuse ecchymosis across the left flank (positive Grey Turner). Peritoneal signs cannot be reliably assessed given obtundation. FAST as above.
- **Pelvis:** Pelvic binder in place. Open wound approximately 4 cm over the left iliac crest with visible cancellous bone; ongoing venous ooze. Perineal blood at the introitus; no gross rectal blood on digital exam; no high-riding prostate applicable. Foley catheter placed after retrograde urethrogram — gross hematuria on return.
- **Back / Spine:** Log-rolled with in-line stabilization. Diffuse abrasions and contusions across the thoracolumbar back. No step-off palpated.
- **Extremities:**
 - **Right lower extremity:** Open comminuted mid-shaft tibia/fibula fracture (Gustilo IIIA) with approximately 8 cm anterior wound and exposed bone. Palpable DP pulse; ABI deferred.
 - **Left lower extremity:** Open comminuted distal tibia/fibula fracture (Gustilo IIIB) with soft-tissue loss over the anteromedial ankle. Palpable but weak DP pulse; capillary refill sluggish. Foot warm.
 - **Upper extremities:** Multiple abrasions and contusions bilaterally. No obvious deformity. Radial pulses 2+ bilaterally.
- **Skin:** Multiple road-rash abrasions across the torso and extremities; grossly contaminated.
- **Neurologic:** As above — GCS 5T, sedated; no localizing response; no spontaneous purposeful movement observed prior to paralytic wearing.

LABORATORY DATA (drawn 02:50 on arrival)

Test	Value	Reference
pH (arterial)	7.19	7.35–7.45
PaCO ₂	34 mmHg	35–45
PaO ₂	188 mmHg	on 100% FiO ₂
Base deficit	–11	
Lactate	6.4 mmol/L	0.5–2.2
Hemoglobin	8.6 g/dL	12.0–15.5
Hematocrit	26.1 %	36–46
Platelets	172 K/ μ L	150–400
INR	1.6	0.9–1.1
PT	18.8 s	
PTT	41 s	
Fibrinogen	178 mg/dL	200–400
Sodium	141 mEq/L	

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Test	Value	Reference
Potassium	4.2 mEq/L	
Chloride	108 mEq/L	
Bicarbonate	15 mEq/L	
BUN	14 mg/dL	
Creatinine	1.1 mg/dL	
Glucose	168 mg/dL	
Ionized Ca	0.98 mmol/L	1.10–1.30
Ethanol	Not detected	
Urine tox	Pending	
β-hCG	Negative	
Type & cross	O-neg started; type-specific pending	

IMAGING SUMMARY

- **CT Head (non-contrast):** Diffuse axonal injury with scattered petechial hemorrhages at the gray–white matter junctions bilaterally, splenium of the corpus callosum, and dorsal midbrain. No large intra- or extra-axial hematoma. Early diffuse cerebral edema with effacement of cortical sulci; basilar cisterns preserved. Small left parietal scalp hematoma correlating with clinical laceration.
- **CT C-Spine:** No acute fracture or malalignment.
- **CT Chest/Abdomen/Pelvis (contrast):**
 - Right-sided flail segment involving ribs 4–9 with right hemopneumothorax; right pleural drain in position.
 - Bilateral pulmonary contusions, right greater than left.
 - Grade IV splenic laceration with active contrast extravasation and moderate hemoperitoneum.
 - No solid-organ injury of the liver, pancreas, or kidneys identified; small left perinephric stranding.
 - **Open pelvic ring disruption:** vertically unstable left-sided injury — displaced fractures of the left superior and inferior pubic rami with associated left sacroiliac joint diastasis; open wound overlying the left iliac crest tracks to the pelvic ring; small pelvic hematoma without active arterial blush on this study.
 - Foley in bladder; retrograde urethrogram (prior series) without frank extravasation but with cystographic contrast tracking suspicious for bladder-dome injury.
- **CT lower extremities:** Bilateral open comminuted tibia and fibula fractures as described; no popliteal vascular cutoff on CTA reformats.
- **CXR (portable):** ETT 3 cm above the carina. Right chest tube in position with re-expansion of the right lung. Right rib fractures and pulmonary contusion.
- **FAST (bedside):** Free fluid in the left upper quadrant; remainder negative.

ASSESSMENT

34-year-old female pedestrian polytrauma status post high-energy auto-vs-pedestrian impact by a transit bus at approximately 30 mph with a fifteen-foot displacement:

1. **Severe traumatic brain injury** — diffuse axonal injury on head CT; GCS 5T on arrival; early cerebral edema.
2. **Open pelvic fracture** — (Gustilo IIIA) — vertically unstable left-sided ring disruption with contamination.
3. **Bilateral open tibia/fibula fractures** — right Gustilo IIIA, left Gustilo IIIB.
4. **Multiple right-sided rib fractures (4–9)** — with **right hemopneumothorax** and flail segment; bilateral pulmonary contusions.
5. **Grade IV splenic laceration** — with active extravasation and hemoperitoneum.
6. **Hemorrhagic shock** — with acute traumatic coagulopathy and lactic acidosis.
7. **Suspected bladder-dome injury** — with gross hematuria.
8. **Multiple abrasions and scalp laceration.**

Prognosis: Guarded. Life-threatening intracranial and intra-abdominal injuries; multi-system involvement; hemodynamic instability on arrival.

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PLAN

- **Disposition:** Direct to Operating Room from trauma bay for emergent exploratory laparotomy with splenectomy versus splenorrhaphy, packing as indicated, and orthopedic stabilization (external fixation of pelvis and bilateral lower-extremity spanning fixators) — combined trauma-surgery/orthopedics team. Post-op admission to Surgical/Neurosurgical ICU.
- **Massive transfusion protocol** — active; 1:1:1 ratio; goal MAP $\geq$ 65 with permissive hypotension pending source control. TXA administered en route to OR.
- **Neurosurgery consult** — placed; ICP monitor (external ventricular drain versus intraparenchymal bolt) to be placed intraoperatively or immediately post-op; maintain CPP $\geq$ 60; head-of-bed 30°; normothermia; seizure prophylaxis with levetiracetam.
- **Orthopedic surgery consult** — placed; combined case with trauma surgery; IV cefazolin + gentamicin per open-fracture protocol; tetanus updated.
- **Urology consult** — placed; further evaluation of suspected bladder injury after primary survey stabilization.
- **Interventional radiology** — on standby for pelvic angio-embolization pending intra-operative findings.
- **Respiratory:** Lung-protective ventilation; sedation with propofol/fentanyl; SAT/SBT deferred while unstable.
- **DVT prophylaxis:** Mechanical (SCDs) at present; chemoprophylaxis deferred given intracranial injury and active bleeding.
- **NPO;** OG tube post-intubation.
- **Family:** Sister Marisela Reyes en route; social work engaged. Family updated by trauma team regarding critical status and guarded prognosis. Patient is currently unable to participate in care decisions given her neurologic status; surrogate decision-making being coordinated with next of kin.
- **Serial exams:** Q1h neuro checks in ICU post-operatively; serial hematocrits; serial ABGs and lactate.

Electronically signed by:

Priya Ramaswamy, MD

Attending, Trauma Surgery

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10/18/2025 04:38